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Beyond clearance: durable skin outcomes with IL-23 inhibition in moderate-to-severe plaque psoriasis

52-week results from LUMEN-1, a Phase 3 trial of nexalumab

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Sample — fictional data. Nexalumab, LUMEN-1, Northbridge Biotherapeutics, the speaker, hospital and congress are invented for portfolio purposes.

AGENDA

Today’s session: four questions, one evidence story

01	**Where do current treatments fall short?** Unmet need in moderate-to-severe plaque psoriasis	5 min
02	**How does nexalumab work?** Selective IL-23p19 blockade	5 min
03	**What did LUMEN-1 show?** Design, efficacy to week 52, subgroups, safety	30 min
04	**What does it mean for practice?** Key takeaways and panel discussion	20 min

Timings are indicative for a 60-minute symposium.

Even on biologics, many patients do not reach complete or lasting clearance

~1 in 2

patients on current biologics do not reach PASI 90 at 1 year*

38%

report that residual plaques still affect daily life (DLQI >1)*

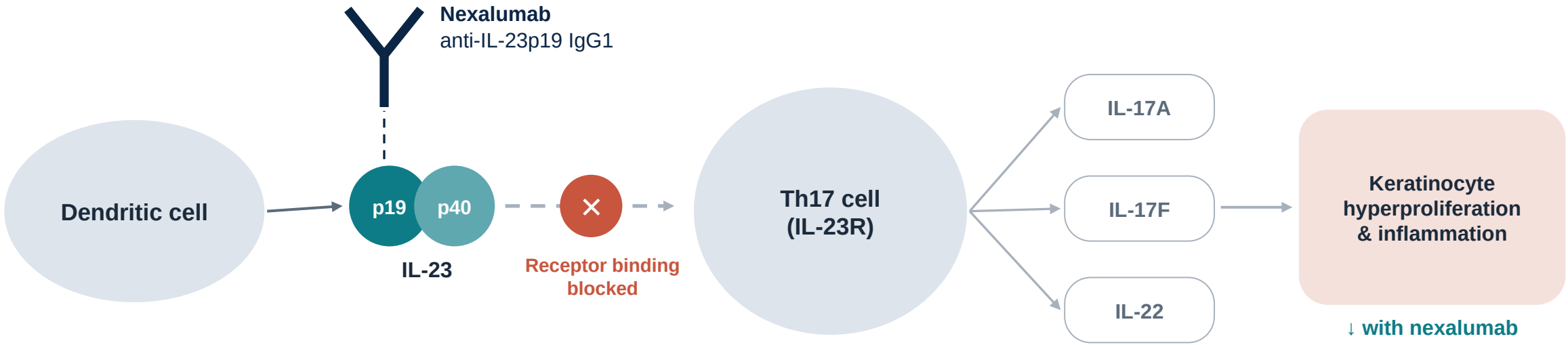
2.4

mean treatment switches in the 5 years after starting a first biologic*

Treatment goal is shifting from ‘better’ to **clear skin that stays clear** — with infrequent dosing and a predictable safety profile

*Illustrative figures from a fictional registry (PSO-REG, n=2,140) created for this sample; not real-world data.
DLQI, Dermatology Life Quality Index; PASI, Psoriasis Area and Severity Index.

Nexalumab binds IL-23p19, switching off the IL-23/Th17 axis upstream



Selective

Targets p19 only, leaving IL-12 (p35/p40) signalling intact

Upstream

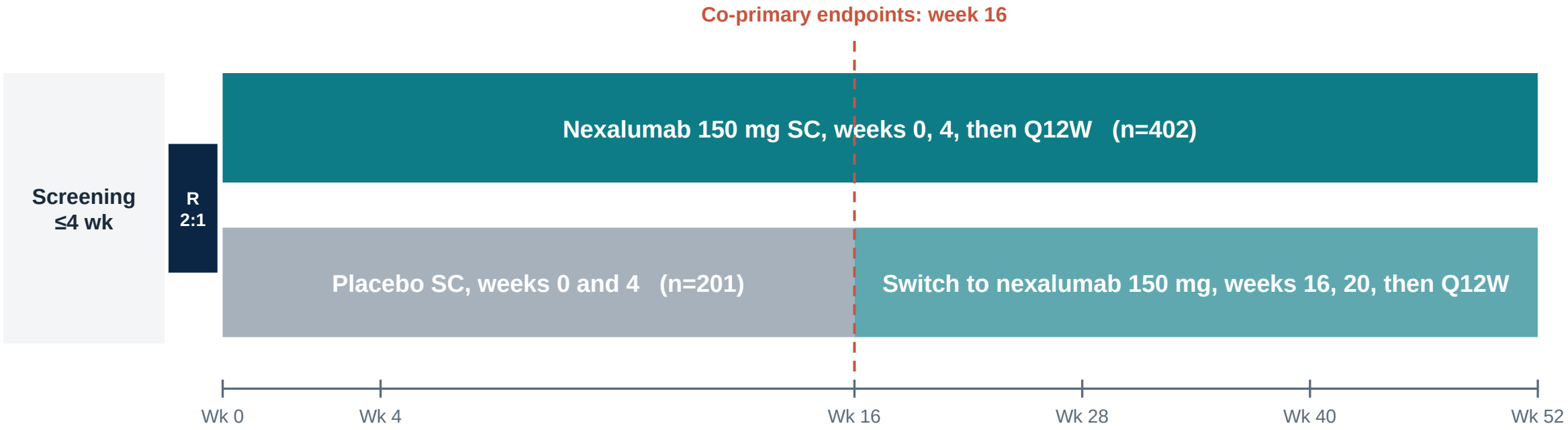
Reduces several downstream effector cytokines at once

Infrequent dosing

150 mg SC at weeks 0 and 4, then every 12 weeks

Simplified schematic. IgG1, immunoglobulin G1; IL, interleukin; IL-23R, IL-23 receptor; SC, subcutaneous; Th17, T helper 17 cell.

LUMEN-1: randomised, double-blind, placebo-controlled, 52 weeks



Key inclusion criteria

- Adults ≥18 years; plaque psoriasis ≥6 months
- PASI ≥12, sIGA ≥3 and BSA ≥10%
- Candidate for systemic therapy or phototherapy

Co-primary endpoints (week 16)

- PASI 90 response
- sIGA 0/1 (clear/almost clear) with ≥2-grade improvement

Key secondary endpoints

- PASI 100 and DLQI 0/1 at week 16
- Maintenance of PASI 90 at week 52
- Safety and tolerability

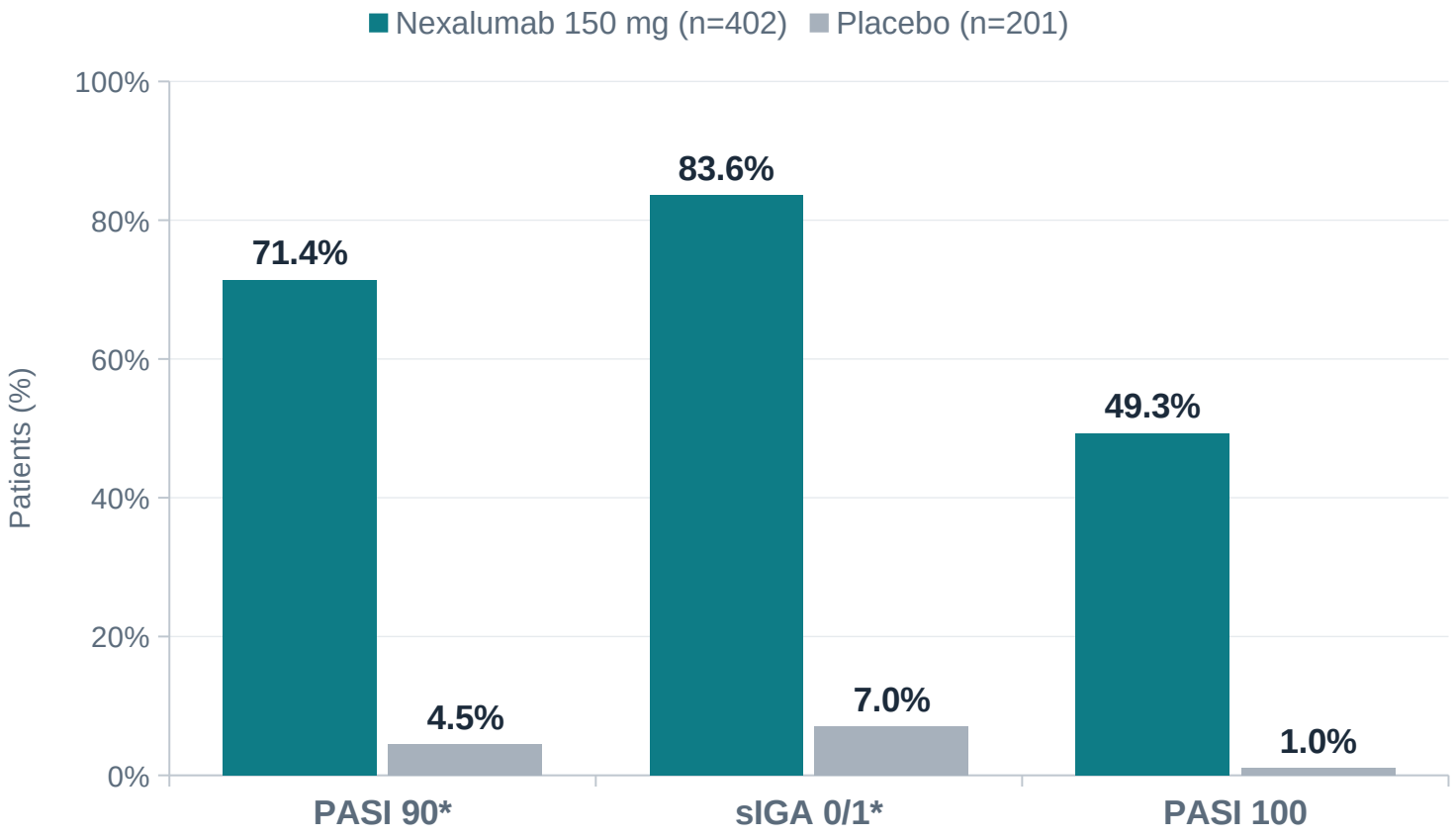
N=603 randomised at 112 sites in 14 countries (fictional). BSA, body surface area; Q12W, every 12 weeks; R, randomisation; sIGA, static Investigator's Global Assessment.

Baseline characteristics were well balanced between treatment arms

Characteristic	Nexalumab 150 mg (n=402)	Placebo (n=201)
Age, years, mean (SD)	45.8 (13.1)	46.3 (12.7)
Male, n (%)	274 (68.2)	135 (67.2)
Body weight, kg, mean (SD)	88.9 (21.4)	89.6 (22.0)
Disease duration, years, mean (SD)	17.2 (11.6)	16.9 (11.9)
PASI, mean (SD)	21.6 (8.3)	21.2 (7.9)
BSA affected, %, mean (SD)	26.4 (15.2)	25.8 (14.9)
sIGA 4 (severe), n (%)	101 (25.1)	49 (24.4)
Prior biologic therapy, n (%)	129 (32.1)	63 (31.3)
Psoriatic arthritis, n (%)	77 (19.2)	37 (18.4)

Full analysis set. SD, standard deviation.

Nexalumab met both co-primary endpoints, with ~7 in 10 patients reaching PASI 90



Responders, n/N

PASI 90

287/402 vs 9/201
Difference 66.9 pp

sIGA 0/1

336/402 vs 14/201
Difference 76.6 pp

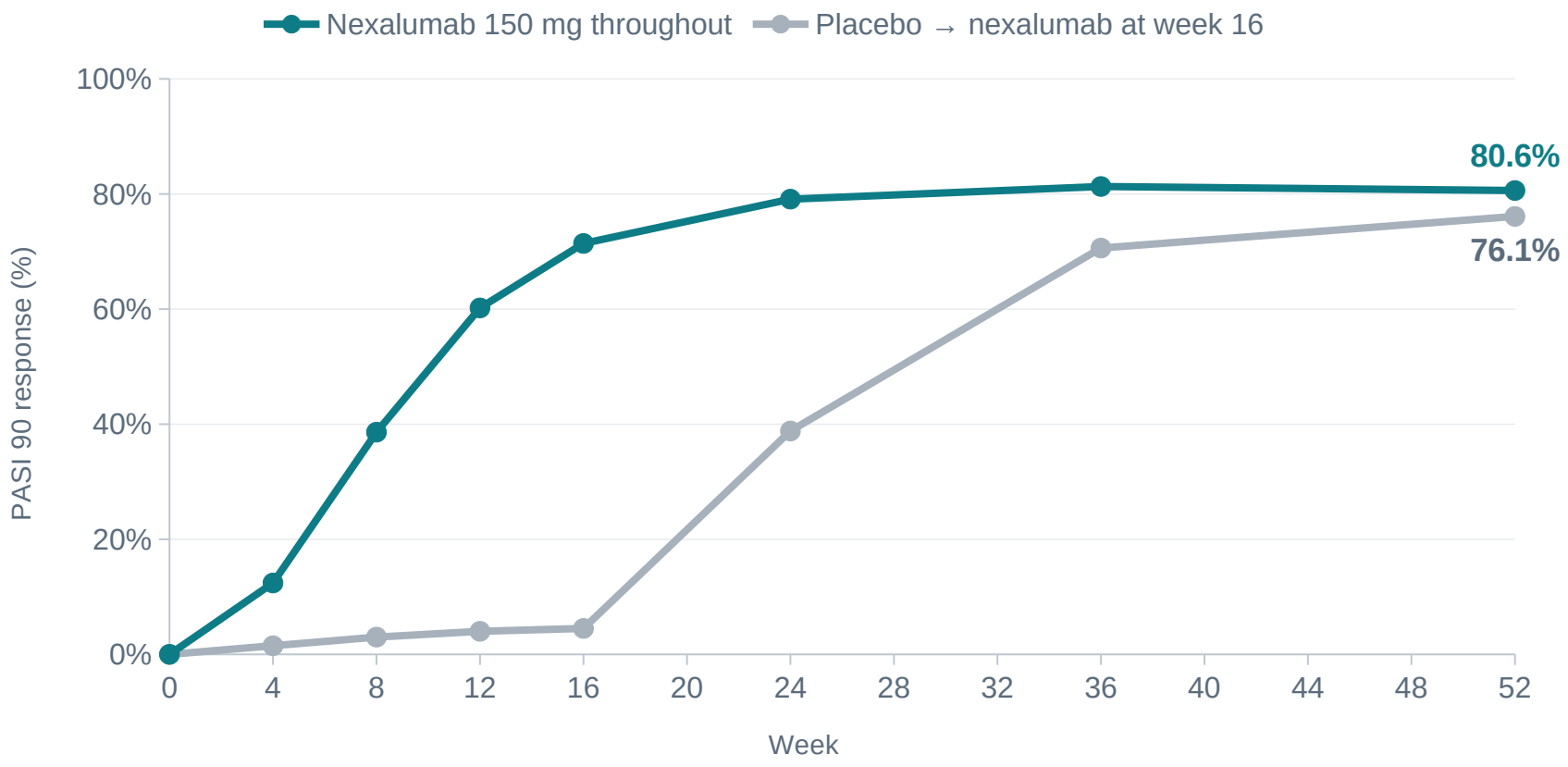
PASI 100

198/402 vs 2/201
Difference 48.3 pp

All comparisons p<0.001

*Co-primary endpoint. Non-responder imputation; Cochran–Mantel–Haenszel test stratified by weight and prior biologic use. pp, percentage points.

PASI 90 responses deepened to week 24 and were maintained through week 52



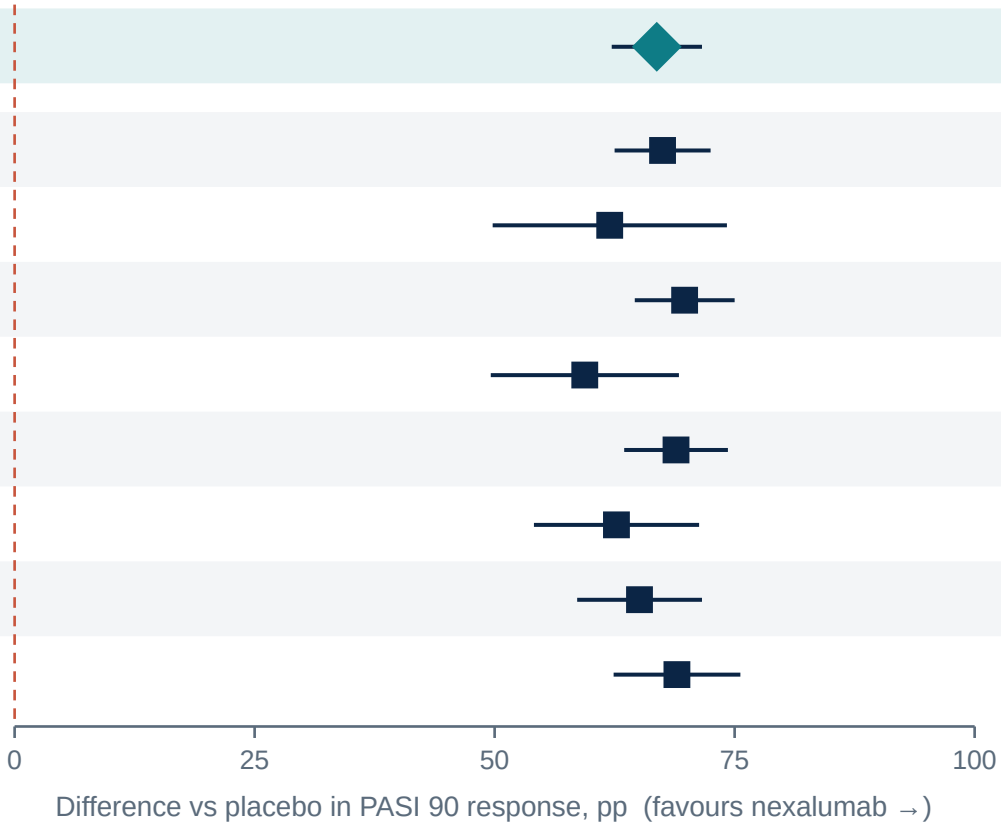
Week 16 → 52
Of week-16 PASI 90 responders on nexalumab, **92.0%** (264/287) maintained PASI 90 at week 52

After switching
Placebo patients approached the continuous-treatment curve within 20 weeks of starting nexalumab

Non-responder imputation. Placebo patients switched to nexalumab at week 16 (week 16 is the last placebo-controlled time point).

The PASI 90 benefit at week 16 was consistent across prespecified subgroups

Subgroup	n (NXB / PBO)	pp (95% CI)
Overall	402/201	**66.9 (62.2, 71.6)**
Age <65 years	352/177	67.5 (62.5, 72.5)
Age ≥65 years	50/24	62.0 (49.8, 74.2)
Weight ≤100 kg	301/150	69.8 (64.6, 75.0)
Weight >100 kg	101/51	59.4 (49.6, 69.2)
Biologic-naïve	273/138	68.9 (63.5, 74.3)
Biologic-experienced	129/63	62.7 (54.1, 71.3)
Baseline PASI ≤20	214/109	65.1 (58.6, 71.6)
Baseline PASI >20	188/92	69.0 (62.4, 75.6)



Prespecified, not powered for formal comparison; 95% CIs are unadjusted. CI, confidence interval; NXB, nexalumab; PBO, placebo; pp, percentage points.

Adverse event rates were similar to placebo, with no new safety signals

Patients with ≥1 event, n (%)	Nexalumab 150 mg (n=402)	Placebo (n=201)
Any TEAE	195 (48.5)	94 (46.8)
Serious adverse event	9 (2.2)	5 (2.5)
AE leading to discontinuation	4 (1.0)	3 (1.5)
Death	0	0
Most common TEAEs (≥3% in either arm)		
Nasopharyngitis	39 (9.7)	16 (8.0)
Upper respiratory tract infection	24 (6.0)	9 (4.5)
Headache	17 (4.2)	7 (3.5)
Events of interest		
Injection-site reaction	11 (2.7)	2 (1.0)
Tinea infection	5 (1.2)	1 (0.5)
Serious infection	2 (0.5)	1 (0.5)
Major adverse cardiovascular event	0	1 (0.5)

Safety set, placebo-controlled period. AE, adverse event; TEAE, treatment-emergent adverse event. No cases of active tuberculosis or inflammatory bowel disease were reported.

LUMEN-1 in four points

1

High response

71.4% PASI 90 and 83.6% sIGA 0/1 at week 16 vs 4.5% and 7.0% with placebo

2

Durable

80.6% PASI 90 at week 52; 92.0% of week-16 PASI 90 responders maintained response

3

Consistent

Benefit seen across age, weight, prior biologic use and baseline severity

4

Well tolerated

AE rates similar to placebo; no new safety signals over 16 weeks

Data are fictional. Interpret alongside the full prescribing information of any real product; nexalumab does not exist.

PANEL DISCUSSION

Questions for the panel

Q1 Which patients in your clinic would you prioritise for an IL-23p19 inhibitor with Q12W maintenance dosing?

Q2 How should PASI 90 durability at 52 weeks weigh against speed of onset when choosing a first biologic?

Q3 What additional evidence would you need for biologic-experienced patients?

Facilitator note: capture points of consensus and divergence for the meeting summary